



POLICY BRIEF | SEPTEMBER 2026

WHO GETS CARE AND WHO DOESN'T?

Closing Migrant Healthcare Gaps Under Climate Stress in Kenya and South Africa

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THE ISSUE IN BRIEF

Executive summary

Climate change is increasing health risks and pressure on public systems. Yet migrants and refugees often face additional barriers to care, including documentation problems, administrative delays, cost constraints and inconsistent frontline decisions.

The central finding: climate stress does not create unequal access by itself. It exposes and intensifies inequalities that already exist.

Kenya

The main point of failure is inconsistent recognition of refugee documentation and uneven administrative practice.

South Africa

Formal rights exist, but access is often shaped by inconsistent application at facility level.

Research at a glance

62	2	JAN-APR	6
participants	countries	2026 fieldwork	study locations

This comparative qualitative study involved 32 participants in Kenya and 30 in South Africa. Kenya field sites were Nairobi, Kakuma, Kalobeyi and Dadaab; South Africa field sites were Johannesburg and Durban.

THE CORE PROBLEM

A static health system serving mobile populations

Health systems are generally designed around stable populations, fixed administrative categories and predictable service demand. Climate-related displacement, cross-border movement and changing settlement patterns challenge those assumptions.

When identity systems, eligibility rules and frontline practice do not adapt, mobility becomes a practical barrier to care. The result is delayed treatment, higher emergency costs and avoidable public-health risk.

Dimension	Kenya	South Africa
System model	Refugee-specific documentation within a mixed public and humanitarian health system.	Constitutional and statutory access through the public health system.
Principal barrier	Inconsistent recognition of refugee documents and administrative status.	Inconsistent interpretation and application of formal rights at facility level.
Where access breaks down	Registration, referrals, insurance access and transitions between humanitarian and public services.	Reception desks, documentation requests, billing decisions and discretionary gatekeeping.
Outcome under climate stress	Existing gaps widen as displacement and service pressure increase.	Uneven implementation becomes more visible as demand and mobility rise.

Equitable access depends on whether rights and documentation are recognised consistently at the point where care is requested.

WHAT THE EVIDENCE SHOWS

Five findings for decision-makers

1 Climate pressure amplifies existing barriers

Extreme weather, displacement and changing patterns of illness increase demand. People already facing administrative or financial barriers are affected first and most severely.

2 Recognition determines access in Kenya

Refugees may hold valid documents yet still face delays when health facilities, referral systems or insurers do not recognise them consistently.

3 Frontline discretion determines access in South Africa

Formal rights are weakened when facility-level decisions vary. Documentation demands, billing practices or gatekeeping can make lawful access depend on individual interpretation.

4 Delayed care is a public-health risk

Postponed treatment can turn manageable conditions into emergencies, increase costs and undermine prevention, surveillance and continuity of care.

5 Parallel services and fragmented planning weaken resilience

Separate humanitarian and public systems may fill short-term gaps, but weak coordination makes transitions difficult and limits system-wide preparedness.

RECOMMENDATIONS

Policy priorities for Kenya

Kenya can strengthen access by making administrative recognition predictable across facilities and by bringing refugee health more fully into national systems.

Connect administrative systems

- Improve interoperability between refugee registration, civil documentation, health facilities and referral systems so valid status can be confirmed without repeated delays.

Expand practical health-insurance access

- Address the operational barriers that prevent eligible refugees and migrants from enrolling, contributing and using cover in practice.

Standardise implementation

- Provide clear, nationally communicated guidance on accepted documentation, registration and referral procedures, supported by staff training and accountability.

Reduce reliance on parallel care

- Build planned transitions and referral pathways between humanitarian providers and the public system while protecting continuity for vulnerable patients.

Implementation test: a person presenting the same valid document should receive the same answer at every facility.

RECOMMENDATIONS

Policy priorities for South Africa

Apply access standards consistently

- Translate formal rights into clear facility procedures, supervision and complaint pathways that reduce discretionary exclusion.

Standardise accepted documents

- Issue practical guidance on which documents are sufficient for registration and care, including cases where papers are missing or being renewed.

Include mobile populations in health planning

- Use service planning assumptions that reflect internal mobility, cross-border movement and climate-linked changes in demand.

Improve emergency coordination

- Align national, provincial and facility responses so migrants and refugees are not excluded during outbreaks, disasters or climate emergencies.

Shared priorities across both countries

- Plan services around changing populations rather than fixed assumptions about residence.
- Develop responsive data systems that protect privacy while showing where access gaps and changing demand are emerging.
- Connect climate, migration and health policy so risks are addressed together rather than through separate plans.
- Resource trusted community organisations as bridges for information, referrals and accountability.

CONCLUSION

From formal entitlement to reliable care

The two country cases reveal different administrative pathways to the same outcome. In Kenya, access is often constrained by whether refugee documentation is recognised. In South Africa, rights may be formally established but inconsistently applied at the frontline.

In both settings, these gaps delay treatment and create unequal access. Under climate stress, the consequences spread beyond individual patients: emergency demand rises, prevention weakens and public systems become less resilient.

A climate-resilient health system must recognise people consistently, apply access rules fairly and plan for mobility as a normal feature of public life.

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This policy brief translates the study findings into practical priorities for public authorities, health-system leaders, humanitarian partners and community organisations.